

PRAXIS UNIVERSITY HOSPITAL

800 Wellness Way, Minneapolis, MN 55415 | (612)555-6608 | www.praxisuh.org

DISCHARGE SUMMARY

Patient Name: Ibrahim Al-Khatib	DOB: August 19, 1974 (Age 51)
MRN: MRN-3291608	Gender: Female
Admission Date: February 10, 2025	Discharge Date: February 22, 2025 (12 days)
Attending Physician: Dr. Reuben Goldstein, MD — Nephrology	Insurance: Humana Choice #HUM-H8-88888
ICU Admission: YES — MICU/SICU	ICU Days: 8 of 12 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute-on-Chronic Renal Failure — Stage 5	N18.6	Acute / Critical
Active Alcohol Use Disorder	F10.20	Chronic / Complicating
Type 2 Diabetes — Uncontrolled (HbA1c 11.2%)	E11.65	Chronic / Complicating

HOSPITAL COURSE

Ibrahim, a 51-year-old female with a history of Active Alcohol Use Disorder and Type 2 Diabetes — Uncontrolled (HbA1c 11.2%), presented with confirmed massive bilateral pulmonary embolism with right ventricular failure and haemodynamic instability (SBP 74 mmHg). The patient was taken emergently to the cath lab where catheter-directed thrombolysis (CDT) with tPA was performed via bilateral pulmonary artery catheters. Haemodynamics stabilised within 6 hours. Follow-up echo at 48 hours showed RV recovery with RVSP 38 mmHg. Anticoagulation was transitioned to rivaroxaban. The patient was monitored in the cardiac ICU for 5 days before transfer to the general cardiology ward. Hypercoagulable workup was initiated and results pending at discharge.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
89/66 mmHg	110 bpm	91% (on 4L O2)	99.9°F	22/min	62 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Cefepime	2g IV	Every 8 hours	Completed in hospital — 7 days
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
G-CSF (Filgrastim)	480 mcg SC	Once daily	Until ANC > 1.0 x10 ³ /μL
Lacosamide	200 mg	Twice daily	Ongoing — neurology follow-up

FOLLOW-UP PLAN

URGENT: Nephrology: Dr. Reuben Goldstein, MD — within 72 hours of discharge (February 25, 2025).

Primary Care: Within 1 week (March 01, 2025). Full labs including BMP, CBC, LFTs.

Home health nursing: Daily vitals and wound check for first 7 days post-discharge.

Inpatient rehabilitation / LTACH: Transfer arranged — starts March 05, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Reuben Goldstein, MD — Nephrology | License #: MN-MD-98012 | Date: February 22, 2025

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